

UPS Request: Please confirm Cigna's acknowledgment of special processing for certain J codes that are excluded under medical and sent to CVS for processing.

The specifics of the special processing are as follows:

1. Claims Process:

1.1. Deny any professional and facility claims associated with all *active* HCPCS codes on the **UPS Site of Care & Drug Benefit Alignment Drug List (attached)** except for the following exclusions:

1.1.1. Inpatient hospital (CMS POS code 21) and emergency room (CMS POS code 23) claims

1.1.2. Claims where Medicare is the primary carrier

Solution/Risks: Systematically, our systems have the ability to either pay or deny by HCPC Code; we are unable to administer by NDC Code. Code J3490 is not a code we can systematically administer with this special provision, it is a miscellaneous code; this is not a code on the original file. We can allow drugs to be set to pay in certain locations via Plan Specs, however, a prepay is required to handle the exceptions, as well as to allow the exceptions to properly apply to the accumulators.

Language will need to be put into Epro that outlines this claim handling, an O-Neutral code will need to be set in Proclaim plan specs to assist in the processing of claims that will apply to the prepay. The indicator will allow the claims to be overridden without negatively impacting the accumulators (ER/IP/OP). It has been confirmed that there are no concerns with this from a national contracting perspective.

Please note, for drugs that we would normally direct members to less intense settings (as opposed to OP Facility), no more steerage will occur. This will apply to a significant number of drugs on the attached list. For any drugs not on the list, we can still provide steerage to more appropriate sites of care.

The claim office has indicated that we can handle the Medicare and COB provision of this benefit.

Ownership/Maintenance of benefits in plan specs/prepay will fall to account team.

From a case management perspective, the risk we run into with this is creating a gap of having members not eligible for case management, since these are now carved out to CVS. Part of the implementation will be working with CVS to determine how their administration of these drugs will trigger our Case Management systems.

Evicore performs reviews of drugs at the Regimen (3-4 drugs that are prescribed together) level; they do have special handling in place now. While we can administer, this does create issues for both providers and members, as they are now required to obtain authorizations from both medical carriers and PBMs.

Differences: In addition to IP hospital and ER, we also need to be able to administer the drugs in the OP facility, as providers will not accept drugs that they do not obtain (they will not accept drugs from outside sources). There are many drugs on the list that are used in transplants; transplants fall under the IP/OP locations, these drugs will not be able to be carved out.

After a call on 10/16/18, it was determined that while not recommended, we can deny claims in the OP Facility; when a claim comes in from one of these locations, it will be denied with a remark code that steers members to CVS (see EOB Message below). Any exceptions would then need to be paid via appeal on the back end.

Costs: \$2500 for set up of the prepay edit, \$1500 annually

Charges for manual processing are \$7.88 per claim; since we do not know projected enrollment, we will need to perform an assessment at the end of the year to determine actual costs.

1.2. Cigna to update claims system pursuant to any drug list updates (will be provided by CVS on a quarterly basis)

Solution: New Drugs will need to be re-evaluated as they are added to the list.

2. EOB message:

2.1. The following explanation of benefit (EOB) message will be applied to the denied claims:

Member/Provider EOB Description: THIS SPECIALTY MEDICATION IS NOT COVERED UNDER THE MEDICAL PLAN. THIS EXPENSE MAY BE COVERED UNDER THE PRESCRIPTION PLAN. PLEASE CONTACT CVS CAREMARK AT 1-855-282-8412 FOR MORE INFORMATION.

Solution: A custom RNC Message form will need to be completed and a CSC Ticket will need to be opened with the Proclaim team. There are no costs for the RNC code, however, there is a 2 month lead time in order to get this implemented.



3. Prior authorization and eligibility process

- 3.1. Cigna will redirect all prior authorization (PA) and eligibility requests meeting the criteria above to CVS Caremark's dedicated line for UPS at 1-855-282-8412.

Solutions/Risks: TruCare will need to put a custom message in place to refer both members and providers to the outside vendor (CVS). This message will need to appear 100% of the time, even if Cigna is responsible for the claim/certification (IP/OP/ER). The soonest that this could be put in place is with the November 30th release. We will also need a Hold Harmless to absolve us of any responsibility for any drugs that were incorrectly administered. Please see RES10083 for instructions on how to set up custom message in TruCare.

Costs: There is a \$7000 OS charge for TruCare setup changes plus \$7000 annual maintenance.

4. Grace fill Process: Each member will be allowed one grace fill (a one-time payment under the medical benefit) at the drug level for any specialty medication that meets the program inclusion criteria above at UPS's discretion. The one-time payment may include multiple dates of service. UPS will provide the approval to Cigna to pay the grace fill per the following process:

- 4.1. If a member with a denied claim escalates to UPS, UPS will forward the member information to the CVS Specialty SPOC team: Megan.Kellenberger@CVSHealth.com, SRX-BISPOCAccount@CVSCaremark.com.
- 4.2. In addition, Cigna will provide a quarterly file identifying any members with paid medical claims to CVS Specialty. CVS Specialty will provide telephonic outreach and education to the member and provider, advising them the drug must be obtained through CVS Specialty under the pharmacy benefit.
- 4.3. Once the transition has been completed, CVS will notify UPS.
- 4.4. UPS will then notify Cigna to approve payment of past medical claims.

Solution: From a Prior Authorization perspective, all PAs need to be treated in the same manner, and we have no way to know if a fill did or did not occur, as we have no way to track the first fill. Our system will look to see if the drug requires review (most on the list will). We will then see that reviews are to be handled by CVS, we will then deny the review and claim, and refer the member/provider to CVS. If it is a drug that we will still administer (not on the list), then we will perform the review. Evicore's system will work in the same manner as Cignas system.

The only way this would work would be via the exception process (see below)

5. Exceptions Process:

- 5.1. The CVS Specialty SPOC team will notify UPS of any exception requests made by a member and/or provider.
- 5.2. UPS will review and approve/deny requests. Pediatric members or drugs infused in conjunction with oncology medications may qualify for an exception, provided UPS approves.
- 5.3. UPS will facilitate benefit exceptions by providing the approval directly to Cigna to allow claims to pay.

Solution: Any exceptions will need to be manually handled via the CSE. The CSE will need to work to have a prior authorization loaded before any claim could be paid. From a serviceability perspective, we have no way to guarantee a level of service on this administration.

OS Charges: This will be completely manual; charges for manual processing are \$7.88 per claim; since we do not know projected enrollment, we will need to perform an assessment at the end of the year to determine actual costs

6. **Data Requirements:** Starting April, 2019, Cigna will generate a quarterly report to identify any claims paid under medical for drugs on the most current UPS Site of Care & Drug Benefit Alignment Drug List. This report will be utilized for member and provider outreach.
 - 6.1. The report will include paid professional and facility medical claims with dates of service incurred within the previous six months pertaining to the full list of HCPCS codes (both ACTIVE and HISTORICAL HCPCS) on the most recent drug list. Note: The full list of HCPCS codes is located on the FIRST TAB of the drug list.
 - 6.2. The paid date range should include the most recent three months and start where the last report ended.
 - 6.3. For example:
 - § The first report in April 2019 will include claims incurred from 1/1/19 through 3/31/19 and paid 1/1/19 through 3/31/19.
 - § The July 2019 report will include claims incurred 1/1/19 through 6/30/19 and paid 4/1/19 through 6/30/19.
 - § The October 2019 report will include claims incurred 4/1/19 through 9/30/19 and paid 7/1/19 through 9/30/19, and so forth.
 - 6.4. All required data elements (highlighted in green) are found on the "CVS Member Provider Outreach File.xls".
 - 6.5. The report must exclude the following program exclusions:
 - § Medicare primary members
 - § Inpatient hospital and emergency room claims
 - § Any claims associated with HCPCS not on the UPS drug list.
 - 6.6. The report will be sent no later than the end of the first month of each quarter (i.e., Jan.31, April 30, July 31, Oct. 31) via encrypted email to: SRX-BISPOCAccount@CVSCaremark.com

Solution: There are a number of reports, including custom reports that can be generated for the account that show this information. Please see RES9566 and RES9636 for information. These reports are the responsibility of the account team.

Account Team Next Steps:

- 1) Account team to work with Kevin Cummings regarding draft Hold Harmless, which accomplishes the following:
 - Memorializes Cigna's agreement to administer a carve-out of certain specialty drugs.
 - Gives Cigna the right to deny a request to administer the carve-out for certain specialty drugs if doing so would not be feasible.
 - Holds Cigna harmless for any liability, such as a mistaken claims payment for a drug carved out from medical benefit administration, that it otherwise may incur in administering the carve-out.
 - Reiterates UPS' responsibility for communicating the arrangement to members.



Medical Site Carve
Out - UPS.docx

- 2) Account team to create custom RNC Code (see above)
- 3) Sales to work with reporting area for custom reports (see above)
- 4) Account Manager works with Underwriting to get the Optional Service Charge assigned appropriately. **\$16,500 for initial setup and \$8500/year for maintenance beginning in year 2. These charges do not include charges for manual processing (tbd).** Sales to complete the form and send it to OSS Coordinators for processing. *There are multiple mailboxes by UW region (i.e. if the UW region is eastern, the form should be sent to OSS Coordinator-Eastern). Since the purpose is to bill the client, the billing option on the form should always indicate "Direct Bill". On the form, be sure to use Service Area "Regional Sales & Underwriting Services", Service Code 260 "Custom Client Charge" and indicate 0000 as the Expense Center. Also, under the additional information you can indicate "Medical Benefit Specialty Pharmacy Carve Out Charges"*



RE: OSC S FORM-0512.xls
ussions - March 4 (284 KB)

- 5) Account manager to Provide final list of approved drugs for carve out, including drug name and HCPC code to client and send to Case installation along with the PBAB approval.

Current list of drugs:



MS002

30713a_30913 FNL S

6) Case Installation:

- Receives approval of carve out, J codes and J code description from Account Manager
- Enters the details of the carve out into ePro under the Injectable Drugs benefit type, including a description of drug being carved out, corresponding HCPC, Pharmacy Vendor Name and phone number

7) NO PGs related to specialty drugs can be administered on this account.

Contracts:

- Receives ePro and reviews for the Injectable Drugs benefit type
- Using the booklet language modifications outlined, the booklet for the corresponding benefit option is manually adjusted in DST by the Contract Analyst



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Benefit Access:

- Receives ePro and reviews for the Injectable Drugs benefit type
- Manually loads the information below into *Notes* section of Benefit Access
This account has opted to carve out the core Pharmacy benefit as well as certain Medical Specialty Medications typically administered under the medical benefit. For details please see ePro.
Pharmacy Benefit Manager: _____
PBM phone number: _____

Pharmacy Service Center:

- Representative checks all areas above to ensure that the appropriate steps have been taken.

Other:

- Provide Melissa Stavely and Gentri Stinson the account numbers, so this account can be added to the Client and HCPC Exclusion list. Adding to the list is manual, and due to the large number of drugs, this could take some time.



CVS Member

Provider Outreach Fil

Calls to discuss:

8/1:

Joyce Olore

Don Lundin

Greg Hensley

Eileen Scott

Donna Roberson

Josh Slopak

8/6:

Brenda Palmer

Devin Dockter
Gary Henshaw
Eileen Scott
Kevin Cummings
Josh Slopak

8/15: Gentry Stinson

8/16: Nina Pederson

8/20: Greg Hensley

Follow up emails:
Anita Manely
Kevin Cummings
Julie Coho/Chris Clough
Donna Roberson
Don Lundin
Gentry Stinson

10/16/18:
Ashley Zubal (IM)
Rachel Fowler (CSS)
Trina Bailey (CSE)
Gary Henshaw (Specialty Pharmacy)
Brenda Palmer (Specialty Pharmacy)
Don Lundin (Claims)
Josh Slopak (PBAB)